



CLINICAL REVIEW FORM TREMFYA

MEDICATION REQUESTED: ☒ TREMFYA Tremfya 100 mg/mL subcutaneous syringe

The submission of this form does not guarantee approval and coverage of the medication.

This form allows the pharmacy benefit manager to review the use of the medication for coverage. Documentation is required.

PATIENT INFORMATION:

Date: 04/29/2026

Patient Name: [REDACTED]

Physician Name: Terry Smith

Physician Phone: 2013507772

Client ID: [REDACTED]

DOB: [REDACTED]

Specialty: rheumatology

Physician Fax: 2082059346

Call Log ID: [REDACTED]

Patient ID: [REDACTED]

PHYSICIAN USE ONLY:

Dose: Quantity: 1.0

Day Supply: 56

Expected Duration of Therapy:

Weight: 210 lb KG / LB (circle)

Height: 162.56 cm CM / IN (circle)

Directions: inject 1 milliliter by subcutaneous route every 8 weeks

Diagnosis: ☐ Plaque Psoriasis (PsO) ☒ Psoriatic Arthritis (PsA) ☐ Ulcerative Colitis (UC) ☐ Crohn's Disease (CD)

☐ Other (specify):

List Diagnosis Codes: L40.50

☒ All Initial Requests: Please answer all questions for initial requests. For reauthorization, go to the "Reauthorization" section.

Will TREMFYA be administered by a healthcare provider or by the patient at home?

☒ Patient ☐ Healthcare provider

Has patient failed methotrexate, cyclosporine, or other immunosuppressant therapy or biologicals for ≥ 90 days?

☒ YES ☐ NO

Must provide documentation of trial length, contraindications, and intolerance.

Was patient evaluated for active or latent tuberculosis (TB) infection by PPD or blood testing within the past year?

☒ YES ☐ NO

If YES, please provide: Results: Negative T SPOT.TB test on 01/05/2026 Test Date:

Is the medication prescribed by or in consultation with an appropriate specialist?

☒ YES ☐ NO

PsO & PsA: Has the patient already received the initial doses of TREMFYA (100mg at Weeks 0 and 4)?

☐ N/A ☒ YES ☐ NO

For Plaque Psoriasis:

Has the patient failed two topical steroids for > 60 days or phototherapy for > 90 days? *Documentation required.*

☐ YES ☐ NO

What is the % BSA coverage and location of the patient's psoriasis? Coverage: % BSA Location:

Has the patient been evaluated for drugs that may cause psoriasiform eruptions (i.e., lithium, beta blocker, antimalarials, etc.)?

☐ YES ☐ NO

For Psoriatic Arthritis:

Has the patient failed two NSAIDs, each for at least 28 days (four weeks)? *Documentation required.*

☒ YES ☐ NO

Please check all that apply to the patient: ☐ Current psoriasis ☐ History of dactylitis ☐ Juxta-articular new bone formation

☐ Negative RF ☐ Personal or family history of psoriasis ☐ Psoriatic nail dystrophy (onycholysis, pitting, and/or hyperkeratosis)

For Ulcerative Colitis and Crohn's Disease:

Has the patient failed or had a contraindication to two of the following medications at max therapeutic doses: corticosteroids, mercaptopurine, azathioprine, or mesalamine? *Documentation of drugs, dates, and duration of trial required.*

☐ YES ☐ NO

Will liver enzymes be monitored at baseline, for at least 16 weeks of treatment, and as needed? *Documentation required.*

☐ YES ☐ NO

Is the patient scheduled to receive or has the patient already received the initial three IV doses of TREMFYA?

☐ YES ☐ NO

If YES, when was the patient's Week 8 dose administered, or when is it scheduled to be administered? Date:

Is the prescribed maintenance dosing for TREMFYA 100mg at Week 16, and every eight weeks thereafter?

☐ YES ☐ NO

If NO, please submit clinical literature supporting prescribed initial maintenance dosing over 100mg every eight weeks for this patient.

☐ All Reauthorization Requests:

Is TREMFYA being administered by a healthcare provider or by the patient at home?

☐ Patient ☐ Healthcare provider

Is the patient at high risk for TB (e.g., hemoptysis, fever, travel to a high burden country, contact with an infected individual, etc.)? If YES, please provide TB test from within the past year: Results: Test Date:

☐ YES ☐ NO

Have chart notes been provided documenting patient stability and improvement on TREMFYA?

☐ YES ☐ NO

****Initial Requests:** Must include chart notes documenting severity of disease state, duration of failed therapies, and labs as listed.

****Reauthorization Requests:** Must include chart notes documenting response to therapy.

CURRENT THERAPY:

Medication(s)/Dose/Duration/Date

(1) Please see addendum on pg 2

FAILED THERAPY:

Medication(s)/Dose/Duration/Date

(2) Please see addendum on pg 2

ADDITIONAL NOTES: (3) Please see addendum on pg 2

Prescriber Signature: Terry Smith

Prescriber NPI: 1508952292

Print Name: Terry Smith

Date: 04/29/2026

The prescribing physician will need to complete this form and fax it, along with supporting documentation (charts/labs/etc.), back to

BeneCard PBF at: 888-830-9450

Only a completed form will be considered for review. This form is for the purpose of obtaining

new or continued prescription treatment for the above member. Release of information via this form does not require member authorization.

BeneCard PBF maintains strict adherence to the protection of member personally identifiable protected health information (PHI) under the HIPAA Act of 1996.

Addendum

(1) NA

(2) Cosentyx Pen 150 mg/mL SC 01/26/2026 – 04/27/2026, NSAIDs trial prior to 10/26/2023

(3) Patient has Psoriatic Arthritis and presents with both cutaneous and joint symptoms. Experiencing pain, stiffness, and swelling. Psoriasis is located on areas such as the low back, bilateral hand and bilateral foot. Tried and failed NSAIDs and Cosentyx. Patient is switching from Cosentyx to Tremfya. Methotrexate, cyclosporine, and acitretin are contraindicated due to regular alcohol consumption. The patient will begin maintenance dosing after completing the loading dose. All alternatives are expected to be less effective, we believe Tremfya is medically necessary as it is superior in addressing both cutaneous and joint symptoms. Negative TB test dated 01/05/2026.

**Jeffrey A. Alper, MD, P.A. -
Vanderbilt Professional Center**

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida
34109-2765



Patient:

Date of Birth:

Date:

PCP:

Historian:

Visit Type:

Office Visit

Document Type:

Chart Note

This 62 year old female presents for Psoriatic Arthritis.

History of Present Illness

1. Psoriatic Arthritis

Pain scale: 6/10. The problem has worsened. The primary symptoms reported include: pain, stiffness and swelling. The patient assesses the interval disease activity as: worsening. The locations affected since last visit are low back, bilateral hand and bilateral foot. Associated symptoms include AM stiffness (1 Hour), inactivity gelling, skin lesion(s) and joint swelling. Pertinent negatives include fever, infection, fatigue, eye symptoms, change in vision, sicca complaints, mouth sores/lesions, chest pain, changing cough, change in exercise tolerance, blood pressure issues, edema, abdominal pain, change in bowel movements, melena or hematochezia, jaundice, urinary symptoms and blood sugar issues. Additional information: She has continued on Cosentyx 150 mg sc q 4 weeks for the past 3 months. she states that her joint pain and her psoriasis have both worsened.

Chronic Conditions Addressed Today

Diagnosis Description	Code	Status	HPI Comments
Arthropathic psoriasis, unspecified	L40.50	Poorly controlled	

Active Medications (Prior to visit today)

Medication Name	Sig Description	Start Date	Stop Date	Refilled	Rx Elsewhere
metoprolol succinate ER 25 mg tablet,extended release 24 hr	take 0.5 tablet by oral route // every other day				Y

aspirin 81 mg tablet	take 1 tablet by oral route	//	Y
	every day		
Zoryve 0.05 % topical cream	as needed	//	Y
Cosentyx Pen 150 mg/mL subcutaneous pen injector	inject 1 milliliter by subcutaneous route every 4 weeks in the abdomen, thigh, or outer area of upper arm (rotate sites)	03/11/2026	N

Allergies

Ingredient	Reaction (Severity)	Medication Name	Comment
NO KNOWN ALLERGIES			

Review of Systems

System	Neg/Pos	Details
Constitutional	Negative	Chills, Fatigue, Fever, Night sweats, Weight gain and Weight loss.
ENMT	Negative	Ear drainage, Hearing loss, Mouth sores, Nasal drainage and SICCA symptoms.
Eyes	Negative	Eye discharge, Eye symptoms, Vision changes and Vision loss.
Respiratory	Negative	Cough, Dyspnea and Wheezing.
Cardio	Negative	Chest pain, Claudication, Edema and Irregular heartbeat/palpitations.
GI	Negative	Abdominal pain, Change in bowel habits, Constipation, Diarrhea, Jaundice, Melena and Vomiting.
GU	Negative	Dysuria, Hematuria, Polyuria (Genitourinary) and Urinary difficulty.
Endocrine	Negative	Cold intolerance, Heat intolerance, Polydipsia, Polyphagia and Polyuria (Endocrine).
Neuro	Negative	Extremity weakness, Gait disturbance, Numbness, Numbness in extremity, Paresthesia, Tingling and Tremors.
Psych	Negative	Anxiety and Depression.
Integumentary	Positive	Skin lesion.
Integumentary	Negative	Pruritus and Rash.
MS	Positive	Back pain, Bone/joint symptoms, Gelling phenomenon, Joint pain, Joint stiffness, Joint swelling, Morning stiffness.
MS	Negative	Joint deformity, Joint tenderness, Low back pain, Muscle cramping, Muscle stiffness, Muscle weakness, Muscular atrophy, Myalgia, Neck pain and Bone fracture.
Hema/Lymph	Negative	Easy bleeding, Easy bruising and Lymphadenopathy.
Allergic/Immuno	Negative	Environmental allergies, Food allergies and Infections.
Reproductive	Negative	Dysmenorrhea, Irregular menses, Menorrhagia and Vaginal discharge.

Vital Signs

Time	BP mm/Hg	Pulse /min	Resp /min	Temp F	Ht ft	Ht in	Ht cm	Wt lb	Wt kg	BMI kg/m2	BSA m2	O2 Sat%
2:51 PM	130/78	97			5.0	4.00	162.56	210.00	95.254	36.05	2.07	91

Disease Activity Measures

Patient Global: 10.0
Pain Score: 8.5
RAPID 3: 20.8

Physical Exam

Exam	Findings	Details
Constitutional	Normal	Level of distress - Normal. Nourishment - Normal. Overall appearance - Normal.
Respiratory	Normal	Auscultation - Normal. Effort - Normal.
Cardiovascular	Normal	Heart rate - Regular rate. Rhythm - Regular. Heart sounds - Normal S1, Normal S2. Murmurs - None.
Vascular	Normal	Pulses - Dorsalis pedis: Normal.
Skin	Normal	Inspection - Normal. Nails - Normal.
Musculoskeletal	*	Hands - Left: joint tenderness, swelling, Right: joint tenderness, swelling. SI - Left: joint tenderness, Right: joint tenderness. Foot - Left: joint tenderness, Right: joint tenderness.
Musculoskeletal	Normal	Gait - Normal. Cervical spine - Good alignment without scoliosis, tenderness, or deformity. Thoracic spine - Good alignment without scoliosis, tenderness, or deformity. Lumbar spine - Good alignment without scoliosis, tenderness, or deformity. Shoulders - No tenderness, swelling, effusion, or limitation to range of motion. Elbows - No tenderness, swelling, effusion, or limitation to range of motion. Wrists - No tenderness, swelling, effusion, or limitation to range of motion. Hips - No tenderness, swelling, effusion, or limitation to range of motion. Ribs - Left: Normal, Right: Normal. Pelvis - Left: Normal, Right: Normal. Knees - No tenderness, swelling, effusion, or limitation to range of motion. TMJ Joints - Normal. AC Joints - Normal. SC Joints - Normal.
Extremity	Normal	No Cyanosis. No edema. Clubbing - Absent.
Neurological	Normal	Level of consciousness - Normal. Cognition - Normal. Memory - Normal. Sensory - Normal. Motor - Normal. Balance & gait - Normal.
Psychiatric	Normal	Orientation - Oriented to time, place, person & situation. Appropriate mood and affect. Good insight. Good judgment.

Follow-up:

Reason	Timeframe	Comments
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follow-up visit with Terry
Lee Smith NP in 3 Months

in 3 Months

follow-up visit with Terry
Lee Smith NP in 3 Months

in 3 Months

Medication Reconciliation
Medications reconciled today.

Medications

PRESCRIBED TODAY:

Medication	Directions
Tremfya 100 mg/mL subcutaneous syringe	inject 1 milliliter by subcutaneous route every 8 weeks

Orders

Labs

Assessment	Test	Status
L40.50	C-REACTIVE PROTEIN	ordered
L40.50	CBC (INCLUDES DIFF/PLT)	ordered
L40.50	COMPREHENSIVE METABOLIC PANEL	ordered
L40.50	SED RATE BY MODIFIED WESTERGREN	ordered

Assessment/Plan

#	Detail Type	Description
1.	Assessment Plan Orders	Arthropathic psoriasis, unspecified (L40.50), chronic, Poorly controlled. C-REACTIVE PROTEIN to be performed, CBC (INCLUDES DIFF/PLT) to be performed, COMPREHENSIVE METABOLIC PANEL to be performed and SED RATE BY MODIFIED WESTERGREN to be performed. Active Medication: Tremfya 100 mg/mL subcutaneous syringe follow-up visit with Terry Lee Smith NP in 3 Months and follow-up visit with Terry Lee Smith NP in 3 Months.
2.	Assessment	Other long term (current) drug therapy (Z79.899).
3.	Assessment	Immunodeficiency due to drugs (D84.821).

Assessment

Assessment Narrative

[Non]erosive:

[Moderate] disease activity based on a DAPSA score = [20.2]. This has [occurred despite] a regimen of [Cosentyx 150 mg sc q 4 weeks.]. Remaining symptoms interfere[the same] with ADL's and QOL's. DMARD labs, ROS, and physical examination suggest medication intolerance: [yes][no]. Extra-articular features present ie. uveitis, enthesitis: [no].

Immunodeficiency:

Tolerating well w/o incident infections.

Plan

Plan Narrative

She will discontinue Cosentyx.

PA for Tremfya 100 mg sc q 8 weeks after loading dose.

Risks and benefits of Tremfya are reviewed.

DMARD labs in [12] weeks.

Avoid tobacco use.

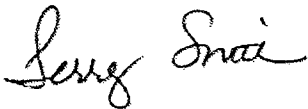
Regular exercise.

Contact office urgently in the event of painful/red eyes.

Pain Management Plan

Pain level:8.5/10.

Terry L. Smith NP



Provider

Terry L. Smith NP

CC Providers:

Document generated by: Terry Smith 04/27/2026 03:37 PM

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida 34109-2765 Fax: 239. 261. 9658 Phone: 239. 799. 2125

Electronically signed by Terry L. Smith NP on 04/27/2026 03:37 PM EDT



**Jeffrey A. Alper, MD, P.A. -
Vanderbilt Professional Center**

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida
34109-2765



Patient:

Date of Birth:

Date:

PCP:

Historian:

Visit Type:

Document Type:

Chart Note

This 62 year old female presents for Psoriatic Arthritis.

History of Present Illness

1. Psoriatic Arthritis

Pain scale: 5/10. The problem has worsened. The primary symptoms reported include: pain, stiffness and swelling. The patient assesses the interval disease activity as: worsening. The locations affected since last visit are low back, bilateral hand and bilateral foot. Associated symptoms include AM stiffness (1 Hour), inactivity gelling, skin lesion(s) and joint swelling. Pertinent negatives include fever, infection, fatigue, eye symptoms, change in vision, sicca complaints, mouth sores/lesions, chest pain, changing cough, change in exercise tolerance, blood pressure issues, edema, abdominal pain, change in bowel movements, melena or hematochezia, jaundice, urinary symptoms and blood sugar issues. Additional information: She continues with joint pain and swelling and dactylitis to the 3rd right DIP joint. She is here today to review diagnostics.

Chronic Conditions Addressed Today

Diagnosis Description	Code	Status	HPI Comments
Arthropathic psoriasis, unspecified	L40.50	Chronic	

Medical/Surgical/Interim History

Reviewed, no change.

Active Medications (Prior to visit today)

[REDACTED]

Medication Name	Sig Description	Start Date	Stop Date	Refilled	Rx Elsewhere
metoprolol succinate ER 25 mg tablet,extended release 24 hr	take 0.5 tablet by oral route // every other day				Y
aspirin 81 mg tablet	take 1 tablet by oral route // every day				Y
Zoryve 0.05 % topical cream	as needed	//			Y

Allergies

Ingredient	Reaction (Severity)	Medication Name	Comment
NO KNOWN ALLERGIES			

Reviewed, no changes.

Family History

Reviewed, no changes.

Social History (Reviewed, updated)

Tobacco use reviewed.

Preferred language is English.

Tobacco use status: Ex-cigarette smoker.

Smoking status: Former smoker.

Tobacco Screening

Patient has used tobacco.

Smoking Status

Type	Smoking Status	Usage Per Day	Years Used	Pack Years	Total Pack Years
Cigarette	Former smoker	40 Cigarettes	37.00	74.00	74.00

Vaping Use

Screened for vaping? Yes
Status: Not a current user
Age started: 48 years
Age stopped: 48 years



Alcohol

There is a history of alcohol use.

Type: Beer. 4 beers consumed monthly.

Patient has had 4 or more drinks in one day: 0 time(s) in the past year.

Home Environment/Safety

The patient has not fallen in the last year.

Review of Systems

System	Neg/Pos	Details
Constitutional	Negative	Chills, Fatigue, Fever, Night sweats, Weight gain and Weight loss.
ENMT	Negative	Dry mouth, Jaw pain, Mouth sores, Mouth ulcers, Nasal sores and SICCA symptoms.
Eyes	Negative	Blurred vision, Double vision, Dry eyes, Eye symptoms and Vision changes.
Respiratory	Negative	Cough, Dyspnea and Wheezing.
Cardio	Negative	Chest pain, Edema, Irregular heartbeat/palpitations and Tachycardia.
GI	Negative	Abdominal pain, Change in bowel habits, Constipation, Decreased appetite, Diarrhea, Jaundice, Melena, Nausea and Vomiting.
GU	Negative	Urinary difficulty.
Endocrine	Negative	Cold intolerance, Heat intolerance and Polydipsia.
Neuro	Negative	Extremity weakness, Headache, Numbness, Numbness in extremity, Paresthesia and Tingling.
Psych	Negative	Anxiety, Depression and Insomnia.
Integumentary	Positive	Skin lesion.
Integumentary	Negative	Hair loss, Hives, Photosensitivity, Pruritus and Rash.
MS	Positive	Bone/joint symptoms, Gelling phenomenon, Joint pain, Joint stiffness, Joint swelling, Morning stiffness, Myalgia.
MS	Negative	Back pain, Joint deformity, Low back pain, Muscle cramping, Muscle stiffness, Muscle weakness, Neck pain and Neck stiffness.
Hema/Lymph	Negative	Easy bleeding and Easy bruising.
Allergic/Immuno	Negative	Allergic rhinitis, Frequent infections and Infections.

Vital Signs

Time	BP mm/Hg	Pulse /min	Resp /min	Temp F	Ht ft	Ht in	Ht cm	Wt lb	Wt kg	BMI kg/m2	BSA m2	O2 Sat%
2:53 PM	114/68	83			5.0	4.00	162.56	209.00	94.801	35.87		98

Comments

Time Comments

2:53 PM Multiple joints pain, swelling and stiffness.

Measured by

Time Measured by

2:53 PM Carmen Ciocan

Physical Exam

Exam	Findings	Details
Constitutional	Normal	Level of distress - Normal. Nourishment - Normal. Overall appearance - Normal.
Respiratory	Normal	Auscultation - Normal. Effort - Normal.
Cardiovascular	Normal	Heart rate - Regular rate. Rhythm - Regular. Heart sounds - Normal S1, Normal S2. Murmurs - None.
Vascular	Normal	Pulses - Dorsalis pedis: Normal.
Skin	Normal	Inspection - Normal. Nails - Normal.
Musculoskeletal	*	Hands - Left: joint tenderness, swelling, Right: joint tenderness, swelling. SI - Left: joint tenderness, Right: joint tenderness. Foot - Left: joint tenderness, Right: joint tenderness.
Musculoskeletal	Normal	Gait - Normal. Cervical spine - Good alignment without scoliosis, tenderness, or deformity. Thoracic spine - Good alignment without scoliosis, tenderness, or deformity. Lumbar spine - Good alignment without scoliosis, tenderness, or deformity. Shoulders - No tenderness, swelling, effusion, or limitation to range of motion. Elbows - No tenderness, swelling, effusion, or limitation to range of motion. Wrists - No tenderness, swelling, effusion, or limitation to range of motion. Hips - No tenderness, swelling, effusion, or limitation to range of motion. Ribs - Left: Normal, Right: Normal. Pelvis - Left: Normal, Right: Normal. Knees - No tenderness, swelling, effusion, or limitation to range of motion. TMJ Joints - Normal. AC Joints - Normal. SC Joints - Normal.
Extremity	Normal	No Cyanosis. No edema. Clubbing - Absent.
Neurological	Normal	Level of consciousness - Normal. Cognition - Normal. Memory - Normal. Sensory - Normal. Motor - Normal. Balance & gait - Normal.
Psychiatric	Normal	Orientation - Oriented to time, place, person & situation. Appropriate mood and affect. Good insight. Good judgment.

Follow-up:

	Reason	Timeframe	Comments
follow-up visit with Terry Lee Smith NP in 3 Months		in 3 Months	

Medication Reconciliation
Medications reconciled today.

Orders

Labs

Assessment	Test	Status
L40.50	C-REACTIVE PROTEIN	ordered
L40.50	CBC (INCLUDES DIFF/PLT)	ordered
L40.50	COMPREHENSIVE METABOLIC PANEL	ordered
L40.50	SED RATE BY MODIFIED WESTERGREN	ordered

Assessment/Plan

#	Detail Type	Description
1.	Assessment Plan Orders	Arthropathic psoriasis, unspecified (L40.50), chronic. C-REACTIVE PROTEIN to be performed, CBC (INCLUDES DIFF/PLT) to be performed, COMPREHENSIVE METABOLIC PANEL to be performed and SED RATE BY MODIFIED WESTERGREN to be performed. follow-up visit with Terry Lee Smith NP in 3 Months.
2.	Assessment	Other long term (current) drug therapy (Z79.899).

Assessment

Assessment Narrative

PsA:

Meets 2006 CASPAR criteria: [yes]. Current symptoms affect ADL's and QOL's. Adequate response to NSAIDS: [no].
Extra-articular features present ie. uveitis, enthesitis: [yes]].PsA:

Meets 2006 CASPAR criteria: [yes].

Plan

Plan Narrative

Recent labs are reviewed, Avise CTD panel shows positive ANA titer 1:80 with remainder of auto antibodies negative, sedrate is 67, CRP is 4.3, HLA B27 is negative, CBC is acceptable, CMP is WNL, Tb spot is negative, Hep B surface antigen is negative, Hep C AB is negative.

Treatment options are discussed with her.

Obtain PA for Cosentyx, risks and benefits are reviewed with her.

She is given samples of Cosentyx and instructed on administration technique and handling of the medication.

See lab orders.

Fall Risk Plan

The patient has not fallen in the last year.

Terry L. Smith NP

A handwritten signature in black ink that reads "Terry L. Smith". The signature is written in a cursive, flowing style.

Provider

Terry L. Smith NP

CC Providers:

Document generated by: Terry Smith 01/26/2026 03:42 PM

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida 34109-2765 Fax: 239. 261. 9658 Phone: 239. 799. 2125

Electronically signed by Terry L. Smith NP on 01/26/2026 03:42 PM EST

A thick, solid black horizontal line used to redact a signature or stamp at the bottom of the page.

Jeffrey A Alper MD PA
6605 Hillway Circle
Naples, FL, 341128754

B

Ordering: Smith, Terry Lee

Performing #: HealthLab

Location: Main Office

Tests Ordered : VENIPUNCTURE (2119881), CBC W/DIFF (2180036), SEDIMENTATION RATE, ESR (2180065), CMP(COMPREHENSIVE METABOLIC PANEL) (2120356), CRP (C-REACTIVE PROTEIN) (2120096), HEPATITIS B SURFACE ANTIGEN (2120923), HEPATITIS C ANTIBODY SCREEN, REFLEX TO CONFIRMATION (2120931), HLA-B27 ANTIGEN (2192944), T-SPOT TB TEST (2190763)

CBC W/DIFF (Collection Date: 01/05/2026 13:22, Status: Final)

<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Absolute Basophils	0.1	10 ³ /uL		0.0-0.3	
Absolute Eosinophils	0.1	10 ³ /uL		0.0-0.6	
Absolute Immature Granulocytes	0.0	10 ³ /uL		0.00-0.10	AVISE DIRECT SEND OUT!

Reference ranges for nonbinary/intersex or unspecified gender patients have not been established. Please refer to the following table for ranges established for cisgender patients and evaluate in the clinical context of the individual patient:

<https://labhandbook.nm.org/genderx>

Test Performed by : CDH LAB

ADDRESS: 25 N Winfield Road ,Winfield Illinois 60190

LAB DIRECTOR: Ajay Patel, MD

Absolute Lymphocytes	1.3	10 ³ /uL		1.0-4.0	
Absolute Monocytes	0.4	10 ³ /uL		0.2-1.0	
Absolute Neutrophils	3.3	10 ³ /uL		1.5-8.0	
Absolute NRBCs	0.0	10 ³ /uL		No reference range established	
Basophils	1.0	%		0.0-2.0	
Eosinophils	2.5	%		0.0-8.0	
HCT	44.0	%		(Based on documented legal sex) 34.0-45.0	
HGB	14.0	g/dL		(Based on documented legal sex) 11.6-15.4	
Immature Granulocytes	0.4	%		No defined	Immature Granulocytes (IG) represents

CMP(COMPREHENSIVE METABOLIC PANEL) (Collection Date: 01/05/2026 13:22, Status: Final)

CRP (C-REACTIVE PROTEIN) (Collection Date: 01/05/2026 13:22, Status: Final)

.....

<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
C-Reactive Protein	4.3	mg/L		0.0-10.0	AVISE DIRECT SEND OUT! Test Performed by : CDH LAB ADDRESS: 25 N Winfield Road ,Winfield Illinois 60190 LAB DIRECTOR: Ajay Patel, MD

HEPATITIS B SURFACE ANTIGEN (Collection Date: 01/05/2026 13:22, Status: Final)

<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Hepatitis B Surface Antigen	Non-reactive			Non-reactive	The test method is electrochemiluminescence immunoassay performed on the Roche Cobas e801. Values obtained with different assay methods by other laboratories cannot be used interchangeably. AVISE DIRECT SEND OUT! Test Performed by : CDH LAB ADDRESS: 25 N Winfield Road ,Winfield Illinois 60190 LAB DIRECTOR: Ajay Patel, MD

HEPATITIS C ANTIBODY SCREEN, REFLEX TO CONFIRMATION (Collection Date: 01/05/2026 13:22, Status: Final)

<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Hepatitis C Antibody	Non-reactive			Non-reactive	Antibodies to HCV Not Detected, does not exclude the possibility of exposure to HCV. The test method is electrochemiluminescence immunoassay performed on the Roche Cobas e801. Values obtained with different assay methods by other laboratories cannot be used interchangeably. AVISE DIRECT SEND OUT! Test Performed by : CDH LAB ADDRESS: 25 N Winfield Road ,Winfield Illinois 60190 LAB DIRECTOR: Ajay Patel, MD

HLA-B27 ANTIGEN (Collection Date: 01/05/2026 13:22, Status: Final)

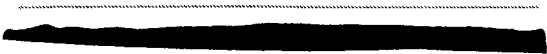
<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Comment HLA B27					HLA-B27 is negative by flow cytometric screening test. Test Performed by: Northwestern Memorial Hospital Laboratory 251 E. Huron St., Chicago, IL 60611 AVISE DIRECT SEND OUT!
HLA B27	Negative				

SEDIMENTATION RATE, ESR (Collection Date: 01/05/2026 13:22, Status: Final)



<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Sedimentation Rate	67	mm/Hour	H	(Based on documented legal sex) 0-30	AVISE DIRECT SEND OUT! Test Performed by : CDH LAB ADDRESS: 25 N Winfield Road ,Winfield Illinois 60190 LAB DIRECTOR: Ajay Patel, MD

T-SPOT TB TEST (Collection Date: 01/05/2026 13:22, Status: Final)



<u>Component</u>	<u>Result</u>	<u>Units</u>	<u>Flag</u>	<u>Range</u>	<u>Comment</u>
Negative Control	Passed				
Panel A Spot Count Corrected for Ne	0				
Panel B Spot Count Corrected for Ne	0				
Positive Control	Passed				

AVISE DIRECT SEND OUT!

Performing Organization Information:
Site ID: V60
Name: Quest Diagnostics TB, LLC-Quest
Diagnostics TB, LL
Address: 5846 Distribution Drive
Memphis, TN 38141-8203
Director: Trivikram Dasu

T-Spot.TB Negative

SeeBelow

Normal Value: Negative
A negative test result does not exclude the possibility of exposure to or infection with Mycobacterium tuberculosis (M. tuberculosis). Patients with recent exposure to TB infected individuals exhibiting a negative T-SPOT.TB result should be considered for retesting within 6 weeks or if other relevant clinical symptoms indicate. Results from T-SPOT.TB testing must be used in conjunction with each individual's epidemiological history, current medical status, and results of other diagnostic evaluations. The T-SPOT.TB test is qualitative and results are reported as positive, borderline or negative, given that the test controls perform as expected. In line with the Centers for Disease Control and Prevention's 2010 recommendation to report quantitative measurements alongside the qualitative result, the laboratory provides spot counts for informational purposes only. The T-SPOT.TB test should not be interpreted as a quantitative test.

VENIPUNCTURE (Collection Date: 01/05/2026 13:22, Status: Final)

[REDACTED]

Jeffrey A. Alper, MD, P.A. -
Vanderbilt Professional Center

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida
34109-2765



Patient:

Date of Birth:

Date:

PCP:

Historian:

Visit Type:

Office Visit

Document Type:

Chart Note

This 62 year old female presents for Psoriatic Arthritis.

History of Present Illness

1. Psoriatic Arthritis

Pain scale: 5/10. The problem has worsened. The primary symptoms reported include: pain, stiffness and swelling. The patient assesses the interval disease activity as: worsening. The locations affected since last visit are low back, bilateral hand and bilateral foot. Associated symptoms include inactivity gelling, skin lesion(s) and joint swelling. Pertinent negatives include fever, infection, fatigue, eye symptoms, change in vision, sicca complaints, mouth sores/lesions, chest pain, changing cough, change in exercise tolerance, blood pressure issues, edema, abdominal pain, change in bowel movements, melena or hematochezia, jaundice, urinary symptoms and blood sugar issues.

HPI Narrative

Diane presents today for evaluation and treatment recommendations regarding suspected psoriatic arthritis. She states that she was diagnosed with psoriasis over 5 years ago. she has psoriasis on her hands, left knee, feet and ankles. She has been treating with topical medications for the psoriasis. She had an MI in January 2025 and states that a couple of months afterwards she developed joint pain and swelling. She has tried Tylenol without relief. She denies any ocular inflammation, stomatitis, serositis, alopecia, sicca symptoms, Raynaud's or inflammatory bowel disease. She reports a family history of RA in a sister and psoriasis and possible PsA in another sister.

Active Medications (Prior to visit today)

Medication Name	Sig Description	Start Date	Stop Date	Refilled	Rx Elsewhere
metoprolol succinate ER	take 0.5 tablet by oral route //				Y
25 mg tablet,extended	every other day				

A handwritten signature in black ink, appearing to be 'Diane', is written over a thick black horizontal line.

release 24 hr			
aspirin 81 mg tablet	take 1 tablet by oral route	//	Y
	every day		
Zoryve 0.05 % topical cream	as needed	//	Y

Allergies

Ingredient	Reaction (Severity)	Medication Name	Comment
NO KNOWN ALLERGIES			

Reviewed, no changes.

Review of Systems

System	Neg/Pos	Details
Constitutional	Negative	Chills, Fatigue, Fever, Night sweats, Middle insomnia, Weight gain and Weight loss.
ENMT	Negative	Dizziness, Dysphagia, Epistaxis, Hearing loss, Hoarseness, Mouth sores, Nasal congestion, Nasal drainage, Posterior auricular pain, anterior auricular pain,, Temporary vision loss, SICCA symptoms, Snoring, Sore throat and Tinnitus.
Eyes	Negative	Blurred vision, Dry eyes, Eye pain, Eye redness, Eye symptoms and Vision changes.
Respiratory	Negative	Cough, Dyspnea and Wheezing.
Cardio	Negative	Chest pain, Edema, Irregular heartbeat/palpitations and Syncope.
GI	Negative	Abdominal pain, Bloating, Change in bowel habits, Constipation, Diarrhea, Jaundice, Melena, Nausea, Dyspepsia and Vomiting.
GU	Negative	Dysuria, Hematuria, Burning w/ urination, Nocturnal urinary incontinence, Polyuria (Genitourinary), Urgency and Urinary difficulty.
Endocrine	Negative	Cold intolerance, Heat intolerance, Hot flashes and Polydipsia.
Neuro	Negative	Difficulty initiating sleep, Headache, Lightheadedness, Memory impairment, Numbness, Numbness in extremity, Paresthesia, Balance problem and Tremors.
Psych	Negative	Anxiety, Depression, Mood swings and Change in sleep pattern.
Integumentary	Positive	Pruritus, Skin lesion, Psoriasis.
Integumentary	Negative	Thick skin, Pigment change and Rash.
MS	Positive	Back pain, Bone/joint symptoms, Gelling phenomenon, Joint pain, Joint stiffness, Joint pain or swelling.
MS	Negative	Joint deformity, Muscle stiffness, Muscle weakness, Myalgia, Neck pain, Bone fracture and shoulder pain.
Hema/Lymph	Positive	Easy bleeding, Easy bruising.
Hema/Lymph	Negative	Lymphadenopathy, Change in hair texture, hormone problems and anemia.
Allergic/Immuno	Negative	Infections.

Vital Signs

Time	BP	Pulse	Resp	Temp F	Ht ft	Ht in	Ht cm	Wt lb	Wt kg	BMI	BSA m2	O2
	mm/Hg	/min	/min							kg/m2		Sat%
2:20 PM	130/82	81			5.0	4.00	162.56	212.00	96.162	36.39		97

Comments

Time **Comments**

2:20 PM Hands , feet , ankles and back pain some times goes up to #8 the pain, mostly in am.

Measured by

Time **Measured by**

2:20 PM Carmen Ciocan

Physical Exam

Exam	Findings	Details
Constitutional	Normal	Level of distress - Normal. Nourishment - Normal. Overall appearance - Normal.
Respiratory	Normal	Auscultation - Normal. Effort - Normal.
Cardiovascular	Normal	Heart rate - Regular rate. Rhythm - Regular. Heart sounds - Normal S1, Normal S2. Murmurs - None.
Vascular	Normal	Pulses - Dorsalis pedis: Normal.
Skin	Normal	Inspection - Normal. Nails - Normal.
Musculoskeletal	*	Hands - Left: joint tenderness, swelling, Right: joint tenderness, swelling. SI - Left: joint tenderness, Right: joint tenderness. Foot - Left: joint tenderness, Right: joint tenderness.
Musculoskeletal	Normal	Gait - Normal. Cervical spine - Good alignment without scoliosis, tenderness, or deformity. Thoracic spine - Good alignment without scoliosis, tenderness, or deformity. Lumbar spine - Good alignment without scoliosis, tenderness, or deformity. Shoulders - No tenderness, swelling, effusion, or limitation to range of motion. Elbows - No tenderness, swelling, effusion, or limitation to range of motion. Wrists - No tenderness, swelling, effusion, or limitation to range of motion. Hips - No tenderness, swelling, effusion, or limitation to range of motion. Ribs - Left: Normal, Right: Normal. Pelvis - Left: Normal, Right: Normal. Knees - No tenderness, swelling, effusion, or limitation to range of motion. TMJ Joints - Normal. AC Joints - Normal. SC Joints - Normal.
Extremity	Normal	No Cyanosis. No edema. Clubbing - Absent.
Neurological	Normal	Level of consciousness - Normal. Cognition - Normal. Memory - Normal. Sensory - Normal. Motor - Normal. Balance & gait - Normal.
Psychiatric	Normal	Orientation - Oriented to time, place, person & situation. Appropriate mood and affect. Good insight. Good judgment.

Follow-up:

Reason	Timeframe	Comments
follow-up visit with Terry Lee Smith NP in 1 Month	in 1 Month	

Medication Reconciliation
Medications reconciled today.

Orders**Labs**

Assessment	Test	Status
D89.89	02-AVISE CTD with reflex to SLE Prognostic when Avise Index Positive	ordered
L40.50	C-REACTIVE PROTEIN	ordered
L40.50	CBC (INCLUDES DIFF/PLT)	ordered
L40.50	COMPREHENSIVE METABOLIC PANEL	ordered
L40.50	HEPATITIS B SURFACE ANTIGEN W/REFL CONFIRM	ordered
L40.50	HEPATITIS C AB W/REFL TO HCV RNA, QN, PCR	ordered
L40.50	HLA-B27 ANTIGEN	ordered
L40.50	SED RATE BY MODIFIED WESTERGREN	ordered
L40.50	T-SPOT	ordered

Other Diagnostic Orders

Status	Order	Code	Timeframe	Appointment	Interpretation
ordered	Sacroiliac Joint X-ray (3+ views)	L40.50			
ordered	Foot X-ray; Complete (3+ views)	L40.50			
ordered	Hand X-ray; Complete (3+ views)	L40.50			
ordered	Lumbar Spine X-ray (including sacrum) (Complete, with oblique views)	L40.50			

Assessment/Plan

#	Detail Type	Description
1.	Assessment	Arthropathic psoriasis, unspecified (L40.50).

Plan Orders C-REACTIVE PROTEIN to be performed, CBC (INCLUDES DIFF/PLT) to be performed, COMPREHENSIVE METABOLIC PANEL to be performed, HEPATITIS B SURFACE ANTIGEN W/REFL CONFIRM to be performed, HEPATITIS C AB W/REFL TO HCV RNA, QN, PCR to be performed, HLA-B27 ANTIGEN to be performed, SED RATE BY MODIFIED WESTERGREN to be performed and T-SPOT to be performed. Foot X-ray; Complete (3+ views) to be performed. on Bilateral, Hand X-ray; Complete (3+ views) to be performed. on Bilateral, Lumbar Spine X-ray (including sacrum) (Complete, with oblique views) to be performed. on Bilateral and Sacroiliac Joint X-ray (3+ views) to be performed. on Bilateral. follow-up visit with Terry Lee Smith NP in 1 Month.

2. Assessment Oth disrd involving the immune mechanism, NEC (D89.89).
Plan Orders 02-AVISE CTD with reflex to SLE Prognostic when Avise Index Positive to be performed.
3. Assessment Inflammatory polyarthropathy (M06.4).
4. Assessment Encounter for testing for latent tuberculosis infection (Z11.7).
5. Assessment Encounter for screening for other viral diseases (Z11.59).
6. Assessment Other long term (current) drug therapy (Z79.899).

Assessment

Assessment Narrative

PsA:

Meets 2006 CASPAR criteria: [yes]]. Current symptoms affect ADL's and QOL's. Adequate response to NSAIDS:][no].
Extra-articular features present ie. uveitis, enthesitis:][no]

Plan

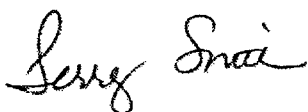
Plan Narrative

Avise CTD panel, sedrate, CRP, HLA B27, Tb spot, Hep C AB and Hep B surface antigen ordered.
X-rays of bilateral hands and feet, SI joints and lumbar spine are ordered.

Fall Risk Plan

The patient has not fallen in the last year.

Terry L. Smith NP



Provider

Terry L. Smith NP

CC Providers:

Document generated by: Terry Smith 12/29/2025 03:07 PM

2340 Vanderbilt Beach Rd., Unit 201, Naples, Florida 34109-2765 Fax: 239. 261. 9658 Phone: 239. 799. 2125

Electronically signed by Terry L. Smith NP on 12/29/2025 03:08 PM EST

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